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Policy Number: C16790-A

Intravenous Bisphosphonates

PRODUCTS AFFECTED

Boniva (ibandronate) IV soln, ibandronate IV soln, pamidronate inj, Reclast (zoledronic acid), zoledronic acid

COVERAGE POLICY

Coverage for services, procedures, medical devices and drugs are dependent upon benefit eligibility as outlined in the member's specific benefit plan. This Coverage Guideline must be read in its entirety to determine coverage eligibility, if any. This Coverage Guideline provides information related to coverage determinations only and does not imply that a service or treatment is clinically appropriate or inappropriate. The provider and the member are responsible for all decisions regarding the appropriateness of care. Providers should provide Molina Healthcare complete medical rationale when requesting any exceptions to these guidelines.

Documentation Requirements:

Molina Healthcare reserves the right to require that additional documentation be made available as part of its coverage determination; quality improvement; and fraud; waste and abuse prevention processes. Documentation required may include, but is not limited to, patient records, test results and credentials of the provider ordering or performing a drug or service. Molina Healthcare may deny reimbursement or take additional appropriate action if the documentation provided does not support the initial determination that the drugs or services were medically necessary, not investigational or experimental, and otherwise within the scope of benefits afforded to the member, and/or the documentation demonstrates a pattern of billing or other practice that is inappropriate or excessive.

DIAGNOSIS:

Osteoporosis and osteoporosis prophylaxis, Bone metastases from solid tumor or multiple myeloma, Hypercalcemia of malignancy, Paget's disease, Osteogenesis imperfecta, Hypercalcemia associated with hyperparathyroidism or ESRD

REQUIRED MEDICAL INFORMATION:

This clinical policy is consistent with standards of medical practice current at the time that this clinical policy was approved. This clinical policy will be reviewed along with state and federal requirements, the benefit being administered, and formulary preferencing. If a drug within this policy receives an updated FDA label within the last 180 days, medical necessity for the member will be reviewed using the updated FDA label information. Coverage will be determined on a case-by-case basis until the criteria can be updated through Molina Healthcare, Inc. clinical governance. Additional information may be required on a case-by-case basis to allow for adequate review. When the requested drug product for coverage is dosed by weight, body surface area or other member specific measurement, this data element is required as part of the medical necessity review. The Pharmacy and Therapeutics Committee has determined that the drug benefit shall be a mandatory generic and that generic drugs will be dispensed whenever available. The Pharmacy and Therapeutics Committee has determined that biosimilars may be preferred.

Drug and Biologic Coverage Criteria

A. OSTEOPOROSIS AND OSTEOPOROSIS PROPHYLAXIS:

1. Documented diagnosis of ONE of the following:
 - a. Postmenopausal osteoporosis
 - b. Postmenopausal woman at risk for a fracture
 - c. Osteoporosis in men
 - d. Man being treated with androgen deprivation therapy for metastatic prostate cancer requiring prophylaxis therapy for osteoporosis [zoledronic acid 4 mg]
 - e. Osteoporosis prevention in postmenopausal women taking aromatase inhibitors for breast cancer [pamidronate, zoledronic acid 4 mg]
 - f. Osteoporosis prevention for members taking prednisone or its equivalent at a dose of > 2.5 mg/day for at least 3 months

AND

2. Documentation of ONE of the following: Member has a diagnosis of esophageal stricture, achalasia, or other severe esophageal dysmotility disorder; OR Member has a history of severe malabsorption making use of oral bisphosphonates ineffective; OR Member has an inability to stand or sit upright for 60 minutes; OR Member has tried and is intolerant to two or more oral bisphosphonates over at least 12 months of therapy.

AND

3. FOR IBANDRONATE: Member has a diagnosis of osteoporosis AND a trial and failure of or labeled contraindication to Reclast (zoledronic acid soln 5MG/100ML)

B. GLUCOCORTICOID-INDUCED OSTEOPOROSIS:

1. Documented diagnosis of glucocorticoid-induced osteoporosis
- AND
2. Documentation of history of prednisone or its equivalent at a dose of > 2.5 mg/day for > 3 months
- AND
3. Request is for ibandronate (Boniva) or zoledronic acid (Reclast)
- AND
4. (a) The member has had a T-score (current or at any time in the past) at or below -2.5 at the lumbar spine, femoral neck, total hip, and/or 33% (one-third) radius (wrist)
- OR
- (b) The member has had an osteoporotic fracture or a fragility fracture of the spine, hip, proximal humerus, pelvis, or distal forearm
- OR
- (c) Fracture Risk Assessment Tool (FRAX) (GC-adjusted) 10-year risk of major osteoporotic fracture score of 20% or greater OR FRAX (GC-adjusted) 10-year risk of hip fracture score of 3% or greater indicating member is at high risk for fracture
- AND
5. Documentation of a trial and failure, serious side effects, or clinical contraindication to one oral generic bisphosphonate therapy
- AND
6. FOR IBANDRONATE: Documentation of trial and failure of or labeled contraindication to Reclast (zoledronic acid soln 5MG/100ML).

C. BONE METASTASES AND MULTIPLE MYELOMA:

1. Documented diagnosis of bone metastases from a solid tumor or multiple myeloma
- AND
2. Request is for pamidronate, ibandronate (Boniva), or zoledronic acid 4 mg
- AND
3. Dose requested aligns with manufacturer recommendation based on creatinine clearance (see Appendix)
- AND
4. FOR PAMIDRONATE: Documentation of a trial and failure of or labeled contraindication to

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zoledronic acid 4 mg

AND

5. FOR IBANDRONATE: Diagnosis of breast cancer and documented trial and failure of or labeled contraindication to zoledronic acid 4 mg

D. HYPERCALCEMIA OF MALIGNANCY:

1. Documented diagnosis of hypercalcemia of malignancy, defined as albumin-corrected serum calcium level greater than 12 mg/dL (3.1 mmol/L)
AND
2. Request is for pamidronate or zoledronic acid 4 mg
AND
3. Dose requested aligns with manufacturer recommendation based on creatinine clearance (see Appendix)
AND
4. FOR PAMIDRONATE: Documentation of a trial and failure of or labeled contraindication to zoledronic acid 4 mg

E. PAGET'S DISEASE:

1. Documented diagnosis of Paget's disease
AND
2. Request is for pamidronate or zoledronic acid (Reclast)
AND
3. Dose requested aligns with manufacturer recommendation based on creatinine clearance (see Appendix)
AND
4. FOR PAMIDRONATE: Documentation of a trial and failure of or labeled contraindication to Reclast (zoledronic acid soln 5MG/100ML)

F. OSTEOGENESIS IMPERFECTA:

1. Documentation of a diagnosis of osteogenesis imperfecta
AND
2. Request is for pamidronate

G. HYPERCALCEMIA ASSOCIATED WITH PRIMARY HYPERPARATHYROIDISM OR END STAGE RENAL FAILURE (INCLUDING MEMBERS WITH SECONDARY HYPERPARATHYROIDISM):

1. Documented diagnosis of hypercalcemia associated with primary hyperparathyroidism or end stage renal failure
AND
2. Request is for pamidronate
AND
3. Documentation that member has been unable to reduce serum calcium with standard care (e.g., restricting calcium-based binders, calcitriol or vitamin D analogs, or calcimimetics)

CONTINUATION OF THERAPY:

A. ALL INDICATIONS:

1. Documentation of positive clinical response to therapy as demonstrated by low disease activity and/or improvements in the condition's signs and symptoms or stabilization of disease
AND
2. Prescriber attests that member will continue to be evaluated and monitored following the FDA labeled monitoring recommendations (Serum creatinine prior to each dose; serum electrolytes, phosphate, magnesium, and hemoglobin/hematocrit should be evaluated regularly; Monitor serum calcium to assess response and avoid overtreatment; In patients with multiple myeloma: Monitor serum creatinine (prior to each dose), serum calcium (regularly); vitamin D levels (intermittently), spot urine sample for albuminuria (every 3 to 6 months; for unexplained

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albuminuria, obtain 24-hour urine collection to assess urinary albumin; reassess every 3 to 4 weeks with 24-hour urine collection for total protein and urine protein electrophoresis until renal function returns to baseline)

AND

3. Dose requested aligns with manufacturer recommendation based on creatinine clearance, if applicable (see Appendix)

AND

4. Prescriber attests to or clinical reviewer has found no evidence of intolerable adverse effects or drug toxicity

AND

5. FOR OSTEOPOROSIS INDICATIONS: Prescriber attests to monitoring the bone mineral density every 1 – 3 years.

DURATION OF APPROVAL:

Initial authorization: 12 months, Continuation of Therapy: 12 months

PRESCRIBER REQUIREMENTS:

Osteoporosis: No requirements

All other indications: Prescribed by, or in consultation with, a board-certified endocrinologist, oncologist, or another applicable specialist. [If prescribed in consultation, consultation notes must be submitted with initial request and reauthorization requests]

AGE RESTRICTIONS:

Osteogenesis imperfecta: No restriction

All other indications: 18 years of age and older

QUANTITY:

Bone Metastasis and Multiple Myeloma:

Boniva or generic equivalent (ibandronate): 6 mg IV every 3 to 4 weeks

Pamidronate: 90 mg IV once every 3 to 4 weeks

Zoledronic acid 4 mg IV every 3 to 4 weeks or every 12 weeks

Hypercalcemia of Malignancy:

Pamidronate: 60 mg or 90 mg IV infusion as a single dose, minimum of 7 days before retreatment

Zoledronic acid 4 mg IV as single dose, minimum of 7 days before retreatment

Osteogenesis Imperfecta:

Pamidronate: 3 mg/kg/cycle, repeated every 4 to 6 months

Paget's Disease:

Reclast or generic equivalent: 5 mg IV as a single dose

Pamidronate: 30mg IV once daily for 3 days

Treatment or Prevention of Glucocorticoid Induced Osteoporosis:

Boniva or generic equivalent (treatment): 2 mg IV every 3 months

Reclast or generic equivalent: 5mg IV every 12 months

Treatment and Prevention of Osteoporosis:

Boniva or generic equivalent (treatment): 3 mg IV every 3 months

Reclast or generic equivalent: 5mg IV x 1 dose for 1 year

Zoledronic acid 4 mg for prevention due to androgen deprivation: 4 mg every 3 to 6 months

Zoledronic acid 4 mg for prevention due to aromatase inhibitors: 4 mg every 6 months

Hypercalcemia due to Hyperparathyroidism:

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Maximum Quantity Limits – Per FDA Label for product

PLACE OF ADMINISTRATION:

The recommendation is that infused medications in this policy will be for pharmacy or medical benefit coverage administered in a place of service that is a non-inpatient hospital facility-based location.

DRUG INFORMATION

ROUTE OF ADMINISTRATION:

Intravenous Infusion

DRUG CLASS:

Bisphosphonates

FDA-APPROVED USES:

Boniva (ibandronate): Indicated for the treatment of osteoporosis in postmenopausal women.

Limitations of Use: Optimal duration of use has not been determined. For patients at low risk for fracture, consider drug discontinuation after 3 to 5 years of use.

Pamidronate: Indicated for the treatment of moderate or severe hypercalcemia associated with malignancy, with or without bone metastases, patients with moderate to severe Paget's disease of bone, for the treatment of osteolytic bone metastases of breast cancer and osteolytic lesions of multiple myeloma in conjunction with standard antineoplastic therapy

Limitations of use: Safety and efficacy of pamidronate disodium in the treatment of hypercalcemia associated with hyperparathyroidism or with other non-tumor related conditions has not been established.

Reclast (zoledronic acid): Indicated for: Treatment and prevention of postmenopausal osteoporosis, Treatment to increase bone mass in men with osteoporosis, Treatment and prevention of glucocorticoid-induced osteoporosis, Treatment of Paget's disease of bone in men and women

Limitations of Use: Optimal duration of use has not been determined. For patients at low risk for fracture, consider drug discontinuation after 3 to 5 years of use.

Zometa (zoledronic acid): Indicated for the treatment of: Hypercalcemia of malignancy, patients with multiple myeloma and patients with documented bone metastases from solid tumors, in conjunction with standard antineoplastic therapy. Prostate cancer should have progressed after treatment with at least one hormonal therapy.

Limitation of use: The safety and efficacy of Zometa has not been established for use in hyperparathyroidism or non-tumor-related hypercalcemia.

COMPENDIAL APPROVED OFF-LABELED USES:

Boniva (ibandronate): For the treatment of hypercalcemia of malignancy, For the prevention of adverse skeletal events due to bone metastases in selected cancer patients (e.g., breast cancer) Pamidronate: For hypercalcemia associated with primary hyperparathyroidism, or hypercalcemia associated with end-stage renal failure including members with secondary hyperparathyroidism, For the treatment of osteogenesis imperfecta

Reclast (zoledronic acid): NA

Zometa (zoledronic acid): For osteoporosis prevention in postmenopausal women taking letrozole for early breast cancer, for osteoporosis prevention in men with prostate cancer receiving androgen deprivation therapy, for the adjuvant treatment of early breast cancer in women with postmenopausal reproductive hormone levels

APPENDIX

APPENDIX:

Renal Impairment dosage adjustments for Multiple Myeloma or Bone Metastases of Solid Tumors

Indications:

NOTE: Zoledronic acid is not recommended in patients with bone metastases who have severe renal impairment; patients with SCr > 3 mg/dl were excluded from clinical trials.

CrCl > 60 ml/min: No dosage adjustment needed

CrCl 50–60 ml/min: Reduce dose to 3.5 mg IV

CrCl 40–49 ml/min: Reduce dose to 3.3 mg IV

CrCl 30–39 ml/min: Reduce dose to 3 mg IV.

CrCl < 30 ml/min: Use not recommended due to lack of clinical data

BACKGROUND AND OTHER CONSIDERATIONS

BACKGROUND:

The World Health Organization (WHO) has defined osteoporosis on the basis of bone mineral density (BMD) measurements to help identify individuals at risk. The bone density Dual X-ray Absorptiometry (DXA) test is one that measures the bone mineral density and compares it to an established norm or standard resulting in a score. The results are compared to the ideal or peak bone mineral density of a healthy 30-year-old adult called a T-score. A T-score is the number of standard deviations (SD) the BMD measurement is above or below the young adult mean bone mineral density.

A T-score between +1 and –1 is considered normal or healthy. A T-score between –1 and –2.5 indicates that you have low bone mass (osteopenia), although not low enough to be diagnosed with osteoporosis. A T-score of –2.5 or lower indicates that you have osteoporosis. The greater the negative number, the more severe the osteoporosis. Bisphosphonate drugs (i.e., zoledronic acid [Reclast™], ibandronate sodium [Boniva]) act to inhibit osteoclast-mediated bone resorption and are used to treat post-menopausal osteoporosis by increasing bone mass. These medications may be administered orally (daily, weekly, or monthly) or by intravenous injection.

CONTRAINDICATIONS/EXCLUSIONS/DISCONTINUATION:

All other uses of ibandronate, pamidronate, and zoledronic acid are considered experimental/ investigational and therefore, will follow Molina's Off-Label policy. Contraindications to IV bisphosphonates include: hypocalcemia and hypersensitivity to the product. Additional contraindication to Reclast (zoledronic acid) includes: patients with creatinine clearance less than 35 mL/min and in those with evidence of acute renal impairment.

Exclusions/Discontinuation:

Ibandronate is not indicated for use in women of reproductive potential. Do not administer ibandronate injection to patients with severe renal impairment (creatinine clearance less than 30 mL/min).

Patients receiving Zometa should not receive Reclast.

Pamidronate disodium may cause fetal harm when administered to a pregnant woman.

Available data on the use of Reclast in pregnant women are insufficient to inform a drug-associated risk of adverse maternal or fetal outcomes. Discontinue Reclast when pregnancy is recognized.

Based on findings from animal studies and its mechanism of action, Zoledronic Acid Injection can cause fetal harm when administered to a pregnant woman.

OTHER SPECIAL CONSIDERATIONS:

Hypocalcemia may worsen during treatment. Patients should be counseled to concurrently take calcium (1000 mg) and vitamin D (400-1200 international units) supplements in conjunction with bisphosphonate therapy. Monitor serum creatinine prior to each dose of IV bisphosphonates.

Standard hypercalcemia-related metabolic parameters, such as serum levels of calcium, phosphate, magnesium, and potassium, should be carefully monitored following initiation of therapy with IV

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bisphosphonates.

Serum calcium, electrolytes, phosphate, magnesium, and CBC, differential, and hematocrit/hemoglobin must be closely monitored in patients treated with pamidronate disodium. Patients who have preexisting anemia, leukopenia, or thrombocytopenia should be monitored carefully in the first 2 weeks following treatment. Patients receiving pamidronate disodium may be at risk for anemia, leukopenia or thrombocytopenia and should have regular hematology assessments.

Cases of osteonecrosis (primarily involving the jaw) have been reported predominantly in cancer patients treated with intravenous bisphosphonates. Many of these patients were also receiving chemotherapy and corticosteroids which may be risk factors for ONJ. Data suggest a greater frequency of reports of ONJ in certain cancers, such as advanced breast cancer and multiple myeloma. The majority of the reported cases are in cancer patients following invasive dental procedures, such as tooth extraction. It is therefore prudent to avoid invasive dental procedures as recovery may be prolonged

CODING/BILLING INFORMATION

CODING DISCLAIMER. Codes listed in this policy are for reference purposes only and may not be all-inclusive or applicable for every state or line of business. Deleted codes and codes which are not effective at the time the service is rendered may not be eligible for reimbursement. Listing of a service or device code in this policy does not guarantee coverage. Coverage is determined by the benefit document. Molina adheres to Current Procedural Terminology (CPT®), a registered trademark of the American Medical Association (AMA). All CPT codes and descriptions are copyrighted by the AMA; this information is included for informational purposes only. Providers and facilities are expected to utilize industry-standard coding practices for all submissions. Molina has the right to reject/deny the claim and recover claim payment(s) if it is determined it is not billed appropriately or not a covered benefit. Molina reserves the right to revise this policy as needed.

HCPCS CODE	DESCRIPTION
J1740	Injection, ibandronate sodium, 1 mg
J2430	Injection, pamidronate disodium, per 30 mg
J3489	Injection, zoledronic acid, 1 mg

AVAILABLE DOSAGE FORMS:

Ibandronate Sodium SOLN 3MG/3ML single-dose prefilled syringe kit
Pamidronate Disodium SOLN 30MG/10ML single-dose vial
Pamidronate Disodium SOLN 6MG/ML single-dose vial
Pamidronate Disodium SOLN 90MG/10ML single-dose vial
Reclast SOLN 5MG/100ML ready-to-infuse solution
Zoledronic Acid CONC 4MG/5ML single-dose vial
Zoledronic Acid SOLN 4MG/100ML
Zoledronic Acid SOLN 5MG/100ML

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SUMMARY OF REVIEW/REVISIONS	DATE
REVISION- Notable revisions: References	Q3 2026
REVISION- Notable revisions: Required Medical Information Duration of Approval Contraindications/Exclusions/Discontinuation Other Special Considerations Available Dosage Forms References	Q3 2025
REVISION- Notable revisions: Required Medical Information Continuation of Therapy Available Dosage Forms References	Q3 2024
REVISION- Notable revisions: Diagnosis Required Medical Information Continuation of Therapy Duration of Approval Prescriber Requirements Place of Administration Drug Class FDA-Approved Uses Contraindications/Exclusions/Discontinuation Coding/Billing Information Available Dosage Forms References	Q3 2023
REVISION- Notable revisions: Products Affected Required Medical Information Continuation of Therapy Prescriber Requirements References	Q3 2022
Q2 2022 Established tracking in new format	Historical changes on file